

**I:** The first question is about familiarity with AI. How familiar are you with AI technologies in general?

**P:** Pretty much familiar, not top level, but I very often use ChatGPT and Gemini, both in my personal and professional life.

**I:** What would you say is your score of familiarity, one being low and ten being high?

**P:** Seven.

**I:** How have you used it in both cases?

**P:** In my personal life, whenever I have a question I just ask ChatGPT straight away. It's easier than using Google. Professionally, whenever I have a question about tests, like how to apply a test or how to understand a score, I use it. First I try to do it myself, and then if I need to check, I use ChatGPT. I've also seen colleagues use it to write reports and documents.

**I:** Has your organization introduced any AI tools or systems, or was that your own decision?

**P:** It's not mandatory. My colleagues started using it first to make sure they were doing things right, and then our bosses paid for Gemini Pro, so now it's another resource we have.

**I:** Do you disclose AI use to clients?

**P:** Sometimes. For example, if we use ChatGPT to generate a text for attention exercises, we might mention it. But not for reports or formal documents.

**I:** So you don't use it for reports, or you just don't disclose it?

**P:** We do not disclose it. I haven't used it for reports myself, but I know colleagues who do and don't mention it.

**I:** Then the next question is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

**P:** I think we need to be aware of how tests are administered and how to interpret results. We need to be professionals and deal with things ourselves. I would only use AI to confirm my hypotheses or to contrast information. I wouldn't rely on it completely.

**I:** If AI wasn't there, would you do this checking with colleagues?

**P:** Yes, with colleagues. Sometimes we discuss things together, and if we still have doubts then we check.

**I:** Can you explain the patient pathway at your center?

**P:** First they come to the center and explain what they've been through. Then there is a screening for physiotherapy, neuropsychology, occupational therapy, and speech therapy. According to their deficits and strengths, the team reaches a consensus on what the rehab process should be. If neuropsychology is needed, which is often the case, they usually have intensive training, with three sessions a week in the first two years. First we do a formal and thorough assessment of attention, memory, executive functions, comprehension, and so on. Then there are different rooms for each area, and when they come they do exercises to strengthen the abilities they've lost.

**I:** Have you considered AI for preliminary screening, or are there parts of that process that could be done by AI?

**P:** I don't think so.

**I:** What about summarizing patient information?

**P:** Not so much for summarizing, but it is useful for understanding medical information, because we're not doctors. Especially for patient history.

**I:** Because most of these clients will have some medical underlying problems causing the cognitive issues. Then the next question is about monitoring and progress tracking. How do you feel about the role of AI in tracking patient progress over time? Do you do any monitoring and progress tracking?

**P:** We do, usually every three or six months, depending on the injury and the case. We administer tests twice: a first test, and then after three to six months we do it again and check the differences. I wouldn't say AI is necessary here, but sometimes if something doesn't make sense, for example if someone has improved in memory but not in attention, or results are even lower than at the beginning, then maybe AI could help us understand that progress. Usually they either improve or stay the same.

**I:** After those three to six months, is that a final test?

**P:** No. We often do follow-ups until we see there is no more improvement, which means the person can probably no longer recover their abilities. After every follow-up we have a meeting with the person and their family, give them all the information we have, and adjust the number of sessions or what we're doing if needed. When we see there is no more improvement, the person usually leaves.

**I:** Is there a basic time frame per patient, or is it very patient dependent?

**P:** Usually around a year and a half to two years, but it's not fixed. Also, if someone has suffered an accident and comes to us a year later, there's not much we can do. The sooner you start, the better. It depends on the patient.

**I:** Then the next question is about administrative and practical support. How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

**P:** I've never used it for reminders or scheduling. I suppose it could be useful, but I just set alarms, which is very fast. For reports, I like to do things myself rather than leaving it to AI. I do my work first and then just check. There is also cognitive data that is more implicit in sessions, so there's a human factor.

**I:** So you mostly see a use case for checking if everything is correct or if something doesn't make sense?

**P:** Yeah. I'm sure that if I enter the data and ask AI the results and explanations will be fine, but I want to do it myself. There is also cognitive data that is more implicit in sessions, so there's a human factor.

**I:** What type of data would that be?

**P:** For example, we sometimes start sessions in the neuropsychology room, but there are also common areas where physiotherapists and occupational therapists work. If we're working on selective attention, we might move the person to a common area to stimulate that. Sometimes we see improvement, but there's no specific test that assesses it. That's more a clinical impression. We test with formal measures, but in the sessions there's something more implicit.

**I:** So you might notice improvement in behavior or memory outside of testing moments.

**P:** Yeah.

**I:** Then the next question is about psychoeducation and between-session tools. What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

**P:** I've used it for both. For psychoeducation, mainly to outline what I can say clearly, because our vocabulary can be very technical and we need to adapt it to the patient's abilities. We also have an Instagram account with psychoeducation posts, and AI helps organize ideas and decide what information goes on each slide. For exercises, we use it a lot to create texts. For example, when working on attention, we use a text and ask the person to cancel specific letters, and creating those texts ourselves takes time. We also use it to create exercises or get ideas for emotion regulation.

**I:** What if AI provides information that contradicts your treatment approach?

**P:** I trust myself. If I think the AI is not right, I would tell it and see how it reacts.

**I:** The question was mostly focused on whether there would be an AI supporting your clients between sessions directly, so they would be interacting with AI themselves rather than you using AI to prepare something for them. That doesn't really apply in your case, right?

**P:** No, I don't think the patient will use it.

**I:** Then the next question is about risks and limitations. What risks or concerns do you have about using AI in clinical work?

**P:** Personal information, mostly. I've seen colleagues use AI to ask about specific exercises or results but entering personal data, and I think that's a very big problem. Every time I use AI I delete any personal data from the text first. Some colleagues haven't, maybe because they forgot or don't find it important. That scares me, because it's happened to me in my personal life too. I've asked ChatGPT something and it seemed to have more information than what I had asked, possibly from other conversations. It's not supposed to keep that information and use it in a different chat. I'm doubtful that they're doing things exactly as they say they are.

**I:** Any other risks or limitations?

**P:** Limitations like transference or countertransference in therapy. When a patient makes you feel a certain way, or reacts in a specific way, and feelings arise. ChatGPT or Gemini cannot feel that, and I think that's very valuable in therapy, especially in psychotherapy.

**I:** Have you thought about issues like bias?

**P:** Because the creators of a specific AI tool are born in a place and have a culture, and every AI does not reply the same thing when you ask a question. Also, if you ask about the interpretation of a specific result, they don't know where you are from, and scores are not the same in the US, in Spain, or in England. I don't think they contrast it with the specific population, so we should be careful there.

**I:** What concerns do you have about transparency or explainability of AI decisions? So if AI gives you an answer, the concern is that you don't really know where it came from, it's not showing its sources, and sometimes it just makes things up. Have you thought about that?

**P:** I usually assume it's based on all the information on the internet and that it trusts reliable sources and rules out information that doesn't follow the scientific method. I hardly ever check where the information came from. But in my personal or student life, if I need something for a project and I'm not sure, I ask about the specific source and check if it's reliable. At work I hardly ever do that.

**I:** Any other limitations or risks you can think of?

**P:** Not that I can come up with now. Maybe if something arises later I'll let you know.

**I:** Then the next question is about client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment?

**P:** I don't think they know we use it most of the time, and they're not very familiar with it. Maybe younger people who are more familiar with it would be okay, but some people might be scared

that we're not doing our job properly because we're using these kinds of tools. They might think we're lazy or not professional enough, or that we lack critical thinking. Usually they don't know, and it depends on the professional. I think I use it properly, only when I need it to make sure I'm right.

**I:** If you told clients you want to use AI in their treatment, how willing do you think they would be, on a scale from one to ten?

**P:** I think if we're transparent and tell them why we want to use it and the benefits, they would be very willing, maybe eight or nine. Some of our patients are pretty old and they're okay with anything, at least in my experience.

**I:** Do you think clients would be more or less engaged with AI-supported treatment?

**P:** It depends on how I use it. The way I use it I don't think it would change their engagement, because I use it for homework and it's basically the same homework, just more easily made. The treatment doesn't really change.

**I:** Then the last question is about professional identity, autonomy, and future conditions. Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

**P:** What I need is to know that the data I enter is safe, just in case I accidentally enter some personal information. As for what should remain strictly human, I don't think we should leave everything to AI and run therapy or rehab exercises strictly at home. Having a person by your side motivating you, understanding how you feel and showing that you actually understand, that's paramount. I've seen that with patients during my internship and now. Some patients, usually because of neurological problems like a brain tumor or a stroke, are depressed or apathetic, and through the therapeutic alliance you can see improvement. I don't think AI can perform that role. Also, having meetings with the family is very important. The way you express yourself, the way you transmit knowledge, your expectations, that can make a difference in how people react to the treatment and to the center. So that can improve engagement.